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Ministry of Health and Family Welfare · Government of India

NATIONAL HEALTH MISSION (NHM) · RMNCH+A CLINICAL GUIDELINES

Comprehensive Antenatal Care (ANC) Field Protocol for Frontline ASHA Workers

DOCUMENT ID: R1-NHM-2026 **EDITION:** Official Field Standard 4.2 **TARGET:** Frontline ASHA Workers

1. Mandatory Antenatal Care (ANC) Schedule

• ANC 1 (Registration): Within 12 weeks of pregnancy. Confirm LMP, calculate EDD, test urine for albumin/sugar, check baseline BP and weight.

• ANC 2: 14 to 26 weeks. Check fetal growth, fundal height, maternal weight gain, and screen for gestational hypertension.

• ANC 3: 28 to 34 weeks. Screen for pre-eclampsia (BP >= 140/90, facial/pedal edema, visual disturbance), repeat hemoglobin estimation.

• ANC 4: 36 weeks to term. Determine fetal presentation and position, arrange institutional delivery plan, prepare emergency transport fund.

2. Essential Diagnostic & Prophylactic Standards

• Iron & Folic Acid (IFA): 1 tablet (100mg elemental iron + 500mcg folic acid) daily for at least 180 days after 1st trimester.

• Calcium Supplementation: 1000mg elemental calcium daily (two 500mg tablets) starting from 14 weeks of gestation.

• Tetanus & adult Diphtheria (Td): Td-1 early in pregnancy followed by Td-2 after 4 weeks (or Td Booster if vaccinated in prior 3 years).

3. Referral Triggers to Primary Health Centre (PHC/CHC)

• Blood Pressure >= 140/90 mmHg on two consecutive readings 15 minutes apart.

• Severe pallor with Hemoglobin < 7.0 g/dL (immediate referral for injectable iron / blood transfusion).

• Unexplained vaginal bleeding or fluid leaking at any gestational age.

Issuing Authority:
National Health Mission Division
Nirman Bhawan, New Delhi

Field Protocol Verification:
Primary Health Centre / Block MO
Mandatory compliance across sub-centres

ASHA Frontline Desk:
Document all actions in MCP Card
Follow 108 Emergency Protocol